

Sample Lumbar Spine MRI Coverage Policy

Document Type: Coverage Policy / Billing and Coding Policy

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Purpose

This synthetic policy describes coverage criteria, billing requirements, documentation expectations, and prior authorization requirements for lumbar spine magnetic resonance imaging. It is designed for testing healthcare content intelligence workflows and does not represent any real payer policy.

Covered Service

Lumbar spine MRI is covered when medically necessary for evaluation of lumbar radiculopathy, suspected spinal stenosis, progressive neurological deficit, trauma, infection, malignancy, or post-surgical complications.

Medical Necessity Criteria

- For uncomplicated low back pain, lumbar MRI is covered after at least six weeks of conservative therapy.
- Conservative therapy may include physical therapy, nonsteroidal anti-inflammatory medication, activity modification, or provider-directed home exercise.
- MRI may be approved before six weeks when progressive neurological deficit, cauda equina symptoms, suspected infection, fracture, malignancy, or severe trauma is documented.
- Repeat lumbar MRI is covered only when results are expected to change clinical management.

Prior Authorization

Prior authorization is required for outpatient lumbar spine MRI. Prior authorization is not required for emergency department imaging, inpatient services, or imaging performed for documented progressive neurological deficit.

Billing and Coding Requirements

Claims must include a valid CPT or HCPCS procedure code, a supported ICD-10 diagnosis code, ordering provider information, rendering provider information, and documentation supporting medical necessity.

Type	Code	Description	Coverage Note
CPT	72148	MRI lumbar spine without contrast	Covered when criteria are met
CPT	72149	MRI lumbar spine with contrast	Covered with infection, tumor, or post-operative indication
CPT	72158	MRI lumbar spine with and without contrast	Requires documented contrast indication
ICD-10	M54.16	Radiculopathy, lumbar region	Supported diagnosis
ICD-10	M48.061	Spinal stenosis, lumbar region	Supported diagnosis
HCPCS	A9575	Injection, gadoterate meglumine	Covered when contrast is medically necessary

Excluded Services

- Screening MRI for nonspecific low back pain without red flags is not covered.
- Routine repeat imaging without a documented change in symptoms is not covered.
- Experimental imaging protocols are excluded unless required under applicable regulation.

Frequency Limit

One lumbar spine MRI is allowed per twelve-month period for the same clinical indication unless there is a documented change in neurological status, new trauma, suspected infection, malignancy, or planned surgical intervention.

Required Documentation

- History and physical examination with date of onset.
- Clinical indication and suspected diagnosis.
- Duration and type of conservative therapy when applicable.
- Neurological examination findings when urgent imaging is requested.
- Prior imaging results when repeat MRI is requested.
- Ordering provider attestation that imaging is expected to affect treatment planning.

Synthetic Contract Terms

For demonstration only, the allowed amount for CPT 72148 is \$750 when performed by an in-network freestanding imaging provider. Member cost share may include a \$50 copay and 20% coinsurance, subject to benefit plan design. These values are synthetic and should not be used for payment.

Decision Examples

Scenario	Expected Policy Outcome	Reason
M54.16, CPT 72148, six weeks therapy, prior authorization present	Approve	Coverage and authorization criteria satisfied
Low back pain, two weeks therapy, no red flags	Deny	Conservative therapy requirement not met
Progressive neurological deficit, no prior authorization	Manual review	Urgent exception may apply

Regulatory and Audit Note

Coverage determinations should comply with applicable federal and state requirements, medical necessity review standards, audit documentation policies, and member benefit language. This synthetic policy contains no PHI and is intended only for software demonstration.